



PATHOLOGY REPORT

Patient Name:**Med. Rec. #:**

DOB:

Gender: F

Ref. Physician:

Patient Address:

Location:

Service: Urology

Billing

Accession #:

Date of Procedure:

Date of Receipt:

Date of Report:

Account #:

Type:

INPATIENT

Additional Copy to:

Ref. Source:

Clinical Diagnosis & History:

Bladder carcinoma.

Specimens Submitted:

- 1: SP: Right ureter; segmental excision
- 2: SP: Left ureter, segmental excision
- 3: SP: Lymph nodes, right common iliac; dissection
- 4: SP: Lymph nodes, right pelvic; dissection
- 5: SP: Lymph nodes, left common iliac; dissection
- 6: SP: Lymph nodes, left pelvic; dissection
- 7: SP: Bladder, uterus, bilateral tubes and ovaries, cervix; resection
- 8: SP: Left ureter #2; segmental excision

DIAGNOSIS:

1. **SP: Right ureter; excision**
- Benign segment of ureter.
2. **SP: Left ureter, excision**
- Segment of ureter with urothelial carcinoma in situ.
3. **SP: Lymph nodes, right common iliac; dissection:**
Lymph Node Dissection:
Metastatic urothelial carcinoma
Number of lymph nodes examined: 4
Number of lymph nodes with metastatic disease: 4
Size of the largest lymph node involved by tumor: 1.7cm.
Size of the largest metastatic focus: 1.2 cm.
Extranodal extension is identified
4. **SP: Lymph nodes, right pelvic; dissection:**
Lymph Node Dissection:
Metastatic urothelial carcinoma
Number of lymph nodes examined: 10
Number of lymph nodes with metastatic disease: 2
Size of the largest lymph node involved by tumor: 1.7cm.
Size of the largest metastatic focus: 1.6 cm.
Extranodal extension is identified
5. **SP: Lymph nodes, left common iliac; dissection:**
Lymph Node Dissection:

1CB-0-3

carcinoma, urothelial, NOS
8/20/3

Site: bladder, NOS C67.9

hw
6/24/11

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIFAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		X
Case is (clin. le):		
Reviewer Initials: <i>hw</i>		
Date Reviewed: 6/20/11		

hw 6/24/11

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Benign lymph nodes
Number of lymph nodes examined: 6

6. SP: Lymph nodes, left pelvic; dissection:

Lymph Node Dissection:

Metastatic urothelial carcinoma
Number of lymph nodes examined: 12
Number of lymph nodes with metastatic disease: 2
Size of the largest lymph node involved by tumor: 1.6cm.
Size of the largest metastatic focus : 1.4 cm.
Extranodal extension is identified

7. SP: Bladder, uterus, bilateral tubes and ovaries, cervix; resection:

Tumor Type:

Urothelial carcinoma with mixed histologic features, including: NOS, squamous and sarcomatoid features

Histologic Grade:

High grade

Pattern of growth of the Non-Invasive component:

Flat (in situ carcinoma)
CIS is extensive

Pattern of growth of the Invasive component:

Infiltrating

Tumor Multicentricity:

Not identified

Bladder Local Invasion:

Perivesical soft tissues

Extravesical Tumor Extension:

Right ureter involved
Left ureter involved
Urethra involved

Vascular Invasion:

Identified

Perineural Invasion:

Identified

Surgical Margins:

Tumor present at left ureteral margin

Non-Neoplastic Mucosa:

Exhibiting chronic cystitis

Female Genital Organs:

The vagina shows no pathologic abnormalities
The endometrium shows atrophy
The myometrium shows leiomyomata
Remaining genital organs are unremarkable

Perivesical Lymph Nodes:

LN Not involved 3
Metastatic carcinoma present in 5 nodes

SURGICAL PATHOLOGY REPORT

Extracapsular extension is present

The Pathologic Stage is (AJCC 2002):
pT3 (Invasion of perivesicle soft tissue)

8. **SP: Left ureter #2; excision:**
- Urothelial carcinoma in situ.

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

*** Report Electronically Signed Out *** 1

Gross Description:

Olca

1) The specimen received fresh, labeled "right ureter" and consists of 0.7x0.6x0.3 cm tan pink issue fragment. At the center, a ureter with 0.3 cm lumen is seen. Entirely submitted for frozen section.

Summary of sections:
FSC- frozen section control

Olca

2) The specimen received fresh, labeled "left ureter" and consists of 1.2x0.5x0.3 cm tan pink issue fragment. At the center, a ureter with 0.4 cm lumen is seen. Entirely submitted for frozen section.

Summary of sections:
FSC- frozen section control

3.) The specimen is received fresh, labeled "Right common iliac lymph node" and consists of a 4.6 x 2.5 x 0.9 cm aggregate of adipose tissue, which is sectioned to reveal four, firm, ovoid, tan-pink lymph nodes, ranging from 0.6 to 2.1 cm greatest dimension. All four lymph nodes are bisected and entirely submitted.

Summary of sections:
BLN – bisected lymph nodes

4. The specimen is received fresh, labeled "right pelvic lymph nodes" is a 7.0 x 4.0 x 1.7 cm aggregate of fibroadipose soft tissue with multiple embedded lymph nodes measuring from 0.5 x 3.0 cm greatest dimension. One of the lymph nodes measuring 1.7 cm in greatest dimension has a firm cut surface. One half of this lymph node is submitted to the TPS. All identified lymph nodes are submitted.

Summary of sections:
FLN- one half of the 1.7 cm firm lymph node
MLN1- one multisectioned lymph node
MLN2- one multisectioned lymph node
BLN- bisected lymph node
LN- lymph nodes

5. The specimen is received fresh, labeled "Left common iliac lymph nodes" and consists of a 3.1 x 1.2 x 0.4 cm aggregate of adipose tissue, which is sectioned to reveal six, firm, irregular to ovoid, tan-pink lymph nodes, ranging from 0.4 to 1.0 cm in greatest dimension. The largest lymph node is bisected and all lymph nodes are entirely submitted.

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Summary of sections:

LN – lymph nodes
BLN – bisected lymph node

6. The specimen is received fresh, labeled "Left pelvic lymph nodes" and consists of an 8.1 x 4.3 x 1.1 cm aggregate of adipose tissue, which is sectioned to reveal 12, firm, irregular to ovoid, tan-pink lymph nodes, ranging from 0.7 to 1.7 cm in greatest dimension. All 12 lymph nodes are bisected and entirely submitted.

Summary of sections:

BLN – bisected lymph nodes

7. The specimen is received fresh, labeled "Bladder, uterus, bilateral ovaries and tubes, cervix". It consist of a 503.0 g, Bladder, uterus, bilateral ovaries and tubes, cervix measuring 16.0 cm from superior to inferior, 10.5 cm laterally and 6.5 cm from anterior to posterior. The bladder measures 6.0 x 5.0 x 2.0 cm, the uterus measures 12.0 x 7.0 x 4.5 cm, the right ovary measures 2.5 x 1.5 x 1.3 cm, the previously ligated right fallopian tube stumps collectively measure 5.0 cm, the left ovary measures 1.8 x 1.5 x 1.0 cm, and the previously ligated left fallopian tube stumps collectively measure 4.5 x 0.3 cm. The portion of vaginal cuff measures 3.0 x 2.0 x 1.8 cm. The distal urethra and the vaginal cuff margin are shaved and submitted. The right ureteric stump measure 4.8 cm in length and 0.5 cm in average diameter and shows no gross lesion, the left ureter stump measures 4.5 cm in length and 0.4 cm in average diameter and shows no gross lesions. The specimen is inked as left – black, right-blue and the bladder is opened anteriorly to reveal a previously incised, 6.0 x 2.0 x 2.0 cm poorly defined, ulcerated lesion, located at the trigone, circumferentially engulfing both ureteral orifices, occupying a portion of the right posterior wall and slightly encroaching on the left posterior wall. On sectioning, the tumor extends to a depth of 1.7 cm, and 2.0 cm to extend into the perivesical fat grossly. The remaining bladder mucosa is congested and tan-purple. The uterus is bivalved to reveal an 8.0 x 6.0 x 0.2 cm triangular endometrial cavity containing a 0.1 cm thick, flat, tan-pink glistening mucosa, devoid of a grossly identifiable lesion. The trabeculated tan-pink myometrium varies from 0.7 to 1.0 cm in thickness and has multiple, rubbery, whorled, homogenous white whorled nodules, ranging from 0.7 to 5.9 cm in greatest dimension. The ovaries and fallopian tubes show no gross lesions. The remaining specimen is submitted for lymph node dissection and any identified lymph nodes are submitted. Tissue has previously been submitted for _____ by Dr. (including right trigone orifice). Gross photographs are taken. Representative sections are submitted.

Summary of sections:

UTHM - urethral margin
RUM - right ureter margin
LUM -left ureter margin
L -lesion, (full thickness cut in half x2)
RUO -right ureteric orifice with tumor
LUO -left ureteric orifice with tumor
LP-left posterior wall with tumor
LA -left anterior wall
RP-right posterior wall with tumor
RA -right anterior wall
TRI -trigone with tumor
DOVE -dome
F -perivesical fat with tumor
EMM -endomyometrium
CX-cervix
DVM -distal vaginal margin, shave
RT – right tube
RO – right ovary
LT – left tube
LO – left ovary
BLN-bisected lymph node
PLN-possible lymph nodes

8. The specimen is received in formalin labeled "left ureter" is a 1.1 cm in length by 0.4 cm in average diameter segment of an unoriented tubular structure with surrounding fat. The specimen is serially sectioned and entirely submitted.

Summary of sections:

U- undesignated

Summary of Sections:

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Part 1: SP: Right ureter; segmental excision (fs)

Block	Sect. Site	PCs
1	fsc	1

Part 2: SP: Left ureter, segmental excision (fs)

Block	Sect. Site	PCs
1	fsc	1

Part 3: SP: Lymph nodes, right common iliac; dissection

Block	Sect. Site	PCs
4	BLN	8

Part 4: SP: Lymph nodes, right pelvic; dissection

Block	Sect. Site	PCs
3	bln	3
1	fln	1
1	ln	1
2	mln1	2
2	mln2	2

Part 5: SP: Lymph nodes, left common iliac; dissection

Block	Sect. Site	PCs
1	BLN	2
1	LN	5

Part 6: SP: Lymph nodes, left pelvic; dissection

Block	Sect. Site	PCs
12	BLN	24

Part 7: SP: Bladder, uterus, bilateral tubes and ovaries, cervix; resection

Block	Sect. Site	PCs
1	BLN	2
2	CX	2
1	DOVE	1
1	DVM	9
2	EMM	3
1	F	1
5	L	5
1	LA	1
1	LO	1
1	LP	1
1	LT	6
1	LUM	1
1	LUO	1
5	PLN	14
1	RA	1
1	RO	1
3	RP	3
1	RT	6
1	RUM	1
1	RUO	1
2	TRI	2
2	UTH	2
1	UTHM	1

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Part 8: SP: Left ureter #2; segmental excision

Block	Sect. Site	PCs
1	U	1

Intraoperative Consultation:

1. SP: Right ureter : Benign.
Permanent diagnosis: Same.
2. SP: Left ureter : Carcinoma in situ.
Permanent diagnosis: Same.